SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Ribociclib + endocrine therapy · metastatic

HR-positive / HER2-negative metastatic breast cancer · NCCP 00525 · version 6 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Ribociclib with an aromatase inhibitor or fulvestrant aims to control advanced HR-positive/HER2-negative breast cancer. The exact endocrine partner depends on previous treatment and menopausal status.

Schedule
Ribociclib is taken once daily on Days 1–21, followed by seven days off, in a 28-day cycle. The endocrine partner continues according to its own prescription. Treatment continues while it is helping and tolerable.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Ribociclib
CDK4/6 INHIBITOR

- ☐ Low white cells and infection
- ☐ Liver injury / abnormal blood tests
- ☐ QT prolongation / heart-rhythm problems
- ☐ Nausea, fatigue and hair thinning
- ☐ Rare serious lung or skin reactions

AI or fulvestrant
ENDOCRINE PARTNER

- ☐ Hot flushes, joint pain and fatigue
- ☐ Bone loss with AI / ovarian suppression
- ☐ Injection-site discomfort with fulvestrant
- ☐ Drug-specific liver and other effects

Discuss the treatment aim, alternatives, expected benefit, serious and lasting risks, fertility/pregnancy precautions, the planned monitoring and who to contact. Review the full longitudinal pathway and confirm which conditional treatments apply.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00525 v6.

SACTCheck v0.83.7
Developed by Dr Paul O'Brien